INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Bevacizumab 15 mg/kg · lung treatment component

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Regimen source
NCCP 00215
Version 7

Day 1 every 21 days

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

How this medicine supports your treatment

Bevacizumab blocks VEGF, a signal that supports blood-vessel growth. In selected non-squamous lung cancers it is added to another treatment to help control cancer; the balance of benefit and risk depends on the partner and tumour features.

TARGETED ANTI-VEGF ANTIBODY

Bevacizumab

Blocks the VEGF blood-vessel growth signal

VEGF stimulates blood-vessel growth → Bevacizumab binds VEGF → Tumour blood-vessel signalling is reduced

Binds VEGF, a signal that promotes blood-vessel growth. Blocking VEGF can reduce the blood supply that supports tumour growth; it also affects normal blood vessels and wound healing.

Report bleeding, severe pain, neurological symptoms or wound problems promptly, and attend blood-pressure and urine checks.

Evidence depends on the treatment partner

Studies have tested bevacizumab with platinum chemotherapy and, separately, with erlotinib in cancers with certain EGFR mutations. The results answer different questions.

These findings concern complete combinations, not bevacizumab alone. They do not rank modern treatment options or predict an individual outcome.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Seek urgent help for sudden severe abdominal pain, significant bleeding or coughing blood, chest pain, breathlessness, a swollen painful leg, a severe headache, confusion, seizures, a change in vision, or a rapidly painful/red spreading wound. Follow the chemotherapy emergency plan for fever or feeling acutely unwell.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and checks

These are relative planning stages, not booked appointments or equal time intervals. Extra blood-pressure, urine, heart or anticoagulation checks depend on individual risk and results.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00215 v7 · official regimen · EMA Avastin product information · current snapshot

THE WHOLE TREATMENT JOURNEY

Bevacizumab 15 mg/kg · lung treatment component: treatment and checks

Bevacizumab 15 mg/kg · lung treatment component

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

First-line non-squamous non-small-cell lung cancer (NSCLC) · platinum or EGFR/erlotinib context

Bevacizumab 15 mg/kg is infused on Day 1 every 21 days. With platinum chemotherapy, the combined phase lasts up to six cycles, then bevacizumab may continue alone. With erlotinib, the combination continues while helpful and tolerated. Progression or unacceptable toxicity can stop either plan earlier; no fixed overall lung-treatment duration is promised.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First infusion	Later cycles	Between visits	Pause or change
Bevacizumab treatment NCCP 00215	Confirm platinum or EGFR/ erlotinib pathway	Day 1 bevacizumab after checks	Every 21 days; review selected partner	Partner has separate schedule	Consultant review before restart or phase change
Platinum or erlotinib phase NCCP 00215	Select one source pathway	Begin separately assessed partner	Platinum: up to six combined cycles	Then bevacizumab alone if appropriate; erlotinib has no automatic switch	Review benefit, toxicity and complete plan
Blood count / kidney / liver NCCP 00215	Baseline tests	Review results	Before each cycle	Extra tests if needed	Review abnormalities and partner treatment

Care / check	Before starting	First infusion	Later cycles	Between visits	Pause or change
Blood pressure NCCP 00215	Measure and control blood pressure (BP)	Before and after treatment	Before and after each treatment	More often if raised or symptomatic	Review control before restarting
Urine protein NCCP 00215	Baseline urine test	Review urine result	Before each cycle	24-hour collection if indicated	Recheck protein before agreed restart
Heart / anticoagulation NCCP 00215	Heart history; echocardiogram (heart ultrasound) (ECHO) if risk; international normalised ratio (blood-clotting test) (INR) if indicated	Review symptoms and medicines	Warfarin INR before cycles once stable	INR weekly until stable; report symptoms	Individual cardiac and anticoagulant review

What each check is for

1

Confirm treatment scope

Before starting and whenever the partner changes

NCCP 00215f: first-line unresectable advanced, recurrent or metastatic non-squamous NSCLC with platinum-based chemotherapy. NCCP 00215g: the corresponding first-line EGFR-activating-mutation setting with erlotinib. This is the bevacizumab component; every partner needs its own prescription, assessment and consent.

Ask / act: Confirm indication, partner, phase and separate assessment.

2

Blood pressure and proteinuria

Baseline, each cycle and extra checks when indicated

These effects can be silent, so attend planned tests even when well.

Ask / act: Agree who books, reviews and acts on each test.

3 Procedures and urgent symptoms

Throughout treatment and treatment breaks

Seek urgent help for sudden severe abdominal pain, significant bleeding or coughing blood, chest pain, breathlessness, a swollen painful leg, a severe headache, confusion, seizures, a change in vision, or a rapidly painful/red spreading wound. Follow the chemotherapy emergency plan for fever or feeling acutely unwell.

Ask / act: Contact the team before procedures or for urgent symptoms.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Tests and review

Before starting: blood count, kidney and liver tests, urine protein, blood pressure and a heart history/examination. Repeat blood count, kidney/liver tests and urine testing before each cycle; check blood pressure before treatment and after it. Extra checks depend on symptoms and results.

Urine results

Protein in the urine can require a 24-hour collection and additional checks. Your team explains how and when to collect it, and reviews whether treatment can continue.

NCCP 00215 v7 · official regimen

Heart and blood-thinning medicines

An ultrasound of the heart may be considered if you have had certain earlier chemotherapy or chest-wall radiotherapy. If you take warfarin, INR tests are usually weekly until stable and then before each cycle; follow your individual plan.

NCCP 00215 v7 · official regimen

Cancer response

Scans and cancer-response checks follow the individual consultant plan. This guide does not promise a fixed scan or repeat heart-ultrasound schedule.

NCCP 00215 v7 · official regimen

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Bevacizumab · Day 1	 **Day 1** intravenous (IV) infusion after checks
Symptoms / extra checks	 **Days 1–21** Report early; follow the agreed blood pressure (BP) and urine plan

Platinum pathway: Bevacizumab every 21 days with the selected platinum chemotherapy for up to six cycles, followed by bevacizumab continuation while helpful and tolerated.

Treatment pauses: Surgery, wound healing, bleeding, blood pressure, urine protein or other problems may change the plan. The team confirms when or whether to restart.

EGFR/erlotinib pathway: Bevacizumab every 21 days alongside separately prescribed daily erlotinib while helpful and tolerated; no automatic six-cycle switch.

Why these bands stay broad: One 21-day bevacizumab reference cycle without calendar dates. The selected partner follows its own schedule; the team reviews the next cycle and any phase change. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00215 v7 · official regimen ↗

There are no guaranteed “safe days”. Educational component reference. The treating team confirms the prescription and tests; the partner requires its own complete assessment and consent.

Treatment and its aim

Who this guide covers

NCCP 00215f: first-line unresectable advanced, recurrent or metastatic non-squamous NSCLC with platinum-based chemotherapy. NCCP 00215g: the corresponding first-line EGFR-activating-mutation setting with erlotinib. This is the bevacizumab component; every partner needs its own prescription, assessment and consent.

NCCP 00215 v7 · official regimen

Schedule and phases

NCCP 00215 v7 · official regimen

Evidence and personal choice

Adding bevacizumab to carboplatin/paclitaxel improved survival in one historical trial. Adding it to erlotinib delayed progression in another trial without a demonstrated overall-survival improvement. These findings concern complete combinations, not bevacizumab alone. They do not rank modern treatment options or predict an individual outcome.

Sandler et al., NEJM 2006: ECOG 4599 carboplatin/paclitaxel with or without bevacizumab · Yamamoto et al., Lung Cancer 2021: JO25567 updated PFS and final OS

Support and urgent symptoms

Seek urgent help for sudden severe abdominal pain, significant bleeding or coughing blood, chest pain, breathlessness, a swollen painful leg, a severe headache, confusion, seizures, a change in vision, or a rapidly painful/red spreading wound. Follow the chemotherapy emergency plan for fever or feeling acutely unwell.



Blood pressure and urine

Take prescribed blood-pressure medicines as agreed and attend urine checks. If a 24-hour collection is requested, confirm how to collect and return it before the next treatment.

NCCP 00215 v7 · official regimen

Surgery, wounds and teeth

Tell the team about planned operations, dental procedures and any wound that is not healing. Bevacizumab can require a treatment break around procedures; the team sets the timing.

Separate partner support

Bevacizumab itself usually needs no routine anti-sickness premedication. Platinum chemotherapy needs its own anti-sickness and other support plan. Erlotinib needs its own tablet, interaction, skin and diarrhoea plan. Keep the complete plan and emergency numbers.

NCCP 00215 v7 · official regimen · EMA Avastin product information · current snapshot · NCCP antiemetic classification v6

Pregnancy and breastfeeding

Discuss fertility and effective contraception before treatment and continue contraception during treatment and for at least six months after the last bevacizumab dose. Do not breastfeed during treatment or for at least six months after the final dose. The partner treatment may require additional precautions.

EMA Avastin product information · current snapshot

Practical support

Blood pressure and urine

Surgery, wounds and teeth

Separate partner support

Pregnancy and breastfeeding

Taking tablets / attending treatment

Practical treatment details

Make treatment fit into daily life

The first infusion is over 90 minutes; if tolerated, the second can be over 60 minutes and later infusions over 30 minutes.

Access and procedures

Bevacizumab needs an intravenous cannula or existing suitable central line; the team decides access.

Support contacts

If a visit or test is missed, contact the team for a revised plan rather than rearranging treatment yourself.

Questions to ask

Which lung pathway am I following?

Ask whether you are receiving bevacizumab with platinum chemotherapy or with erlotinib for an EGFR-mutated cancer. Their partner schedules, checks and phase changes differ.

When does treatment change?

The platinum combination is given for up to six cycles, after which bevacizumab can continue alone if appropriate. The erlotinib pathway has no automatic six-cycle switch. The team reviews benefit, side effects and your priorities.

What happens if urine protein or blood pressure rises?

The team may repeat tests, treat blood pressure or pause bevacizumab. Severe problems may mean stopping it. Do not restart after a pause without an agreed plan.

What if I need an operation or dental work?

Tell the oncology team before the procedure. Bevacizumab can slow healing, and the team must coordinate any treatment break and check healing before restarting.

Explain the plan back: Which lung pathway applies, when are bevacizumab and partner treatments due, when are blood-pressure and urine checks, and which symptoms need urgent help?

Treatment scope · NCCP 00215 Lung context only. Confirm whether the platinum pathway or EGFR/erlotinib pathway applies. This guide does not supply the partner dose or replace its safety assessment.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Bevacizumab lung component · support plan

Active medicines: Bevacizumab

Emetogenic risk: Minimal for bevacizumab only

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Bevacizumab itself usually needs no routine anti-sickness premedication. Platinum chemotherapy needs its own anti-sickness and other support plan. Erlotinib needs its own tablet, interaction, skin and diarrhoea plan. Keep the complete plan and emergency numbers.
- No routine bevacizumab antiemetic prophylaxis; assess partner chemotherapy emetic risk separately.
- Anti-diarrhoeal medicines only if required under local policy.
- Maintain blood pressure (BP)/urine, procedure/wound and urgent-contact plans.
- At the platinum-to-bevacizumab continuation change, review and stop or adapt partner-specific support only on the prescriber's instruction. Erlotinib support continues according to its own plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Treatment scope · National Cancer Control Programme (NCCP) 00215 Lung context only. Confirm whether the platinum pathway or EGFR/erlotinib pathway applies. This guide does not supply the partner dose or replace its safety assessment.

NCCP 00215 · VERSION 7

Bevacizumab 15 mg/kg · lung treatment component

Bevacizumab 15 mg/kg is infused on Day 1 every 21 days. With platinum chemotherapy, the combined phase lasts up to six cycles, then bevacizumab may continue alone. With erlotinib, the combination continues while helpful and tolerated. Progression or unacceptable toxicity can stop either plan earlier; no fixed overall lung-treatment duration is promised.

Bevacizumab with a separately prescribed partner · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Head / vision

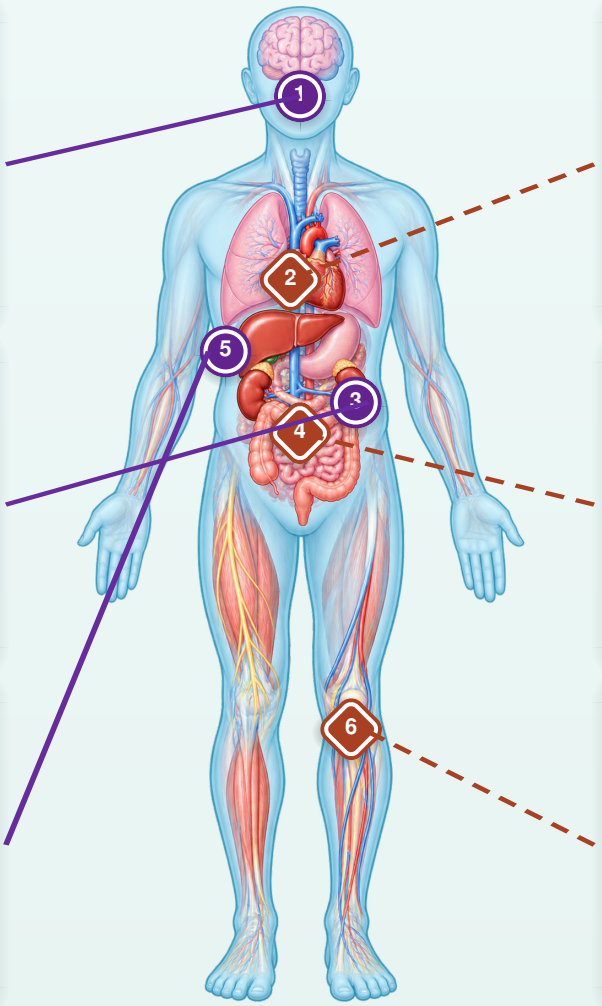
Bevacizumab

Severe headache, confusion, seizures or a change in vision need urgent help.

2 Heart / blood vessels

Bevacizumab

Attend blood-pressure checks; chest pain, breathlessness or a swollen painful leg need urgent help.



3 Kidneys / urine

Bevacizumab

Urine checks look for protein. Report new swelling, foamy urine or reduced urine.

4 Abdomen / bowel

Bevacizumab

Sudden severe abdominal pain needs urgent assessment.

5 Skin / wounds

Bevacizumab

Report bleeding or wounds that are painful, red, spreading or slow to heal.

6 Leg / clots

Bevacizumab

A new swollen painful leg or sudden breathlessness needs urgent help.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Seek urgent help for sudden severe abdominal pain, significant bleeding or coughing blood, chest pain, breathlessness, a swollen painful leg, a severe headache, confusion, seizures, a change in vision, or a rapidly painful/red spreading wound. Follow the chemotherapy emergency plan for fever or feeling acutely unwell. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

Medicine-specific symptom	No concern	Present / manageable	Affects sleep or daily activities — contact team	When / what changed?
Headache / blood-pressure symptoms	☐	☐	☐	
Bleeding / bruising	☐	☐	☐	
Foamy urine / swelling	☐	☐	☐	
Abdominal pain / wound symptoms	☐	☐	☐	

Checks: expected, booked and reviewed are different

Test / review	Due window	Who books it?	Who reviews the result?	Outcome / next action
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____
_____	_____	_____	_____	_____

Missed tablets / injection changes / questions / support needed:

Seek urgent help for sudden severe abdominal pain, significant bleeding or coughing blood, chest pain, breathlessness, a swollen painful leg, a severe headache, confusion, seizures, a change in vision, or a rapidly painful/red spreading wound. Follow the chemotherapy emergency plan for fever or feeling acutely unwell. Emergency symptoms: 112 / 999.

Carry your treatment details

Treatment scope · NCCP 00215 Lung context only. Confirm whether the platinum pathway or EGFR/erlotinib pathway applies. This guide does not supply the partner dose or replace its safety assessment.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

Bevacizumab 15 mg/kg · lung treatment component

NCCP 00215 v7

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Prescriber / next review: _____

Contact plan

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
Emergency: 112 / 999.

Own hospital / contact: _____

Seek urgent help for sudden severe abdominal pain, significant bleeding or coughing blood, chest pain, breathlessness, a swollen painful leg, a severe headache, confusion, seizures, a change in vision, or a rapidly painful/red spreading wound. Follow the chemotherapy emergency plan for fever or feeling acutely unwell.

https://sactcheck.com/patient/00215/lung/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **Current NCCP regimen supportive-care section**
   https://healthservice.hse.ie/documents/6940/00215_Bevacizumab_15.pdf
2. **EMA Avastin product information · current snapshot**
   https://www.ema.europa.eu/en/documents/product-information/avastin-epar-product-information_en.pdf
3. **Sandler et al., NEJM 2006: ECOG 4599 carboplatin/paclitaxel with or without bevacizumab**
   https://pubmed.ncbi.nlm.nih.gov/17167137/
4. **Yamamoto et al., Lung Cancer 2021: JO25567 updated PFS and final OS**
   https://pubmed.ncbi.nlm.nih.gov/33279874/
5. **NCCP antiemetic classification v6**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf